

--SUMMARY--

Decision No. 631/23

18-Sep-2025

V.Patel - M.Moreau - J.Mandoko

- Osteoarthritis (knee)

On November 25, 2020, the worker, a library technician, had a slip and fall. The issue under appeal was whether the worker had initial entitlement for left knee, right hip or right thigh injuries.

The Panel denied the appeal.

The worker's left knee osteoarthritis was a pre-existing condition and it was not caused by the November 2020 accident. The aggravation basis policy provides guidelines for determining the extent to which an aggravation injury contributes to subsequent disability. Medical evidence must establish, on a balance of probabilities, that the workplace injuring process has, in a material way, advanced the pathology of the pre-existing condition.

The Panel found, on a balance of probabilities, that the November 2020 accident did not make a significant contribution to the worker's left knee, right hip or right thigh condition. There was no diagnosis for the worker's right hip and right thigh after the November 2020 accident.

The worker's left knee osteoarthritis was not compatible with the mechanism of injury. The workplace injury did not in a material way, advance the pathology of the pre-existing condition. It is more likely than not that, given the worker's age and weight, the worker's osteoarthritis would have resulted in symptoms of pain regardless of the accident.

18 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4425s]
- MEDICAL DISCUSSION PAPERS CONSIDERED: Osteoarthritis
- TRIBUNAL DECISIONS CONSIDERED: Decision No. 2077/19, 2020 ONWSIAT 33 refd to; Decision No. 1592/01, 2001 ONWSIAT 2556 refd to

Style of Cause:
Neutral Citation: 2025 ONWSIAT 1107



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 631/23

BEFORE

V. Patel : Vice-Chair
M.P. Moreau : Member Representative of Employers
J. Mandoko : Member Representative of Workers

HEARING INFORMATION

Date: February 28, 2025
Location: Toronto
Format: Oral by videoconference
Post-hearing activity: Completed on June 3, 2025

DATE OF DECISION September 18, 2025

NEUTRAL CITATION 2025 ONWSIAT 1107

DECISION(S) UNDER APPEAL: Appeals Resolution Officer (ARO) M. Haughton decision dated March 14, 2022

APPEARANCES

For the worker: N. Sheppard, Union Representative
For the employer: M. Allen, Paralegal
Interpreter: Not applicable
Tribunal Counsel Office: Not applicable

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue, 7th Floor
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**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

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REASONS

1. Introduction

- [1] The worker appeals a decision of an Appeals Resolution Officer (ARO) of the Workplace Safety and Insurance Board (WSIB) dated March 14, 2022.

2. Issues

- [2] The issue under appeal is whether the worker has initial entitlement for left knee, right hip or right thigh injuries.

3. Background

- [3] On November 25, 2020, the then 68 year old worker, a library technician, had a slip and fall. On April 16, 2021 the Entitlement Adjudicator denied initial entitlement for claimed injuries to his left knee and right hip/thigh. On June 3, 2021, on reconsideration the Eligibility Adjudicator upheld the denial. The worker objected and on March 14, 2022 the ARO confirmed the denial of initial entitlement for these injuries. The worker appealed to the Tribunal and the issue is before us.

4. Law and policy

- [4] Since the worker claimed to be injured in 2020, the *Workplace Safety and Insurance Act, 1997*, S.O. 1997, c. 16, Schedule A (WSIA), applies to this appeal. All legislative references in this decision refer to the WSIA, as amended, unless otherwise stated.
- [5] Section 13(1) of the WSIA provides a worker who sustains a personal injury by accident arising out of and in the course of her employment has entitlement to benefits.
- [6] Decisions of the Workplace Safety and Insurance Appeals Tribunal (WSIAT) apply the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance (*Decision No. 280*, 1987 CanLII 1996 (ON WSIAT)). It does not need to be the only contributing factor (*Decision No. 915*, 1987 CanLII 1258 (ON WSIAT)).

- [7] The standard of proof in workers' compensation proceedings is the balance of probabilities as modified by the benefit of the doubt provisions in the WSIA (*Decision No. 2096/18R*, 2019 ONWSIAT 1482). As stated in section 124(2) of the WSIA, the benefit of the doubt is resolved in favour of the claimant where it is not practicable to decide an issue because the evidence for and against it is approximately equal in weight.
- [8] The WSIB stated that the following policy packages, Revision #9, apply to the subject matter of this appeal: Packages 300 and 241.
- [9] The Panel considered these policies when deciding the issue in the appeal, as required by section 126(1) of the WSIA. All policies referred to are from the WSIB's Operational Policy Manual (Policy), unless otherwise stated.

5. Analysis and conclusion

- [10] The appeal is denied for the reasons set out below.
- [11] We considered the legislation, relevant Board policy, the documentation on file, the worker's testimony and the parties' representatives' submissions in making our decision. While we have considered all of the evidence and submissions we have only referenced what is most relevant to our analysis.
- [12] Operational Policy Manual Document No. 11-01-01, "Adjudicative Process," states that an allowable claim must have five points: an employer, a worker, a personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. Operational Policy Manual Document No. 11-01-01 provides guidelines for determining proof of accident.
- [13] There is no dispute that there is an employer and a worker. Of concern are whether there is proof of accident, a personal work-related injury, and compatibility of diagnosis to accident history.
- [14] The worker testified that he was hired as a library technician for the employer in October 2013 and remained in the same position when he stopped working in

July 2024, which is when he retired. The worker testified that he had three falls while he worked for this employer. In 2016, the worker was walking up icy concrete steps of the school, and he went down on both knees, but primarily on his left. The initial fall produced no immediate pain; it showed up a couple of weeks later. Next, in 2018, the worker had a second fall. There was snow and ice in the workplace parking lot and he fell on both knees, mostly his left. He went to the doctor and got physiotherapy over 10-12 weeks. The last appointment was cancelled because of the COVID-19 pandemic. The worker testified that after his second injury he asked to avoid schools with stairs and the accommodation was granted by the employer. The worker then had a third fall in November 2020. After the second fall and extenuated by the third fall in the November 2020 accident, he can't climb stairs but he could before. The worker testified that at the high school where he worked after the November 2020 accident there were two floors but he rarely went up them and would use his cane and go up slowly. The worker testified that prior to the November 2020 accident, he briefly used a walker when he had severe pain and couldn't use a cane.

- [15] The worker testified that he had no serious problems with his knees immediately before November 2020. He had occasional pain on either side but primarily in the left knee. He also mentioned pulling of muscles and the arthritis in his hips flaring up prior to the November 2020 accident.
- [16] On November 25, 2020, the date of the accident in this claim, the worker testified that he was walking down the corridor of his school when he went down on the wet floor. After falling he got up with great difficulty; students held his shoulders to get him up. His left knee was impacted on the concrete floor. His right side was pulled and was painful right away and then the pain went away. His knee pain came and went. The accident was witnessed by others and the worker reported it right away to the employer. The worker went back to his regular duties at work. He was very cautious because he had fallen two times before. The worker testified that he thinks the fall irritated his left knee. After the fall, he had constant or chronic pain; for the rest of 2020 he used a cane. He had throbbing in his knee but he still went about. Before the accident the throbbing in his knee came and went. This time his left knee slowly just got worse; the more he used

it the more it pained him; the pain correlated to activity. His knee got worse from his fall in November 2020 to March 2021.

- [17] The worker testified that he used to swim and do weights which had helped him but after the pandemic he no longer swam and lifted weights. Prior to November 2020 he used to walk and swim. The worker testified that he was taking several medications for pain and inflammation for his knee prior to and after the date of accident. The worker testified that he had relief from treatment. The worker testified to the employer's representative's questions that he did two physiotherapy treatment periods; one for 10 weeks and one for three weeks prior to the November 2020 accident but he did not remember the dates and the name of the physiotherapist.
- [18] The worker stated that the physiotherapy helped him. Some days he feels pain in his knee and others he didn't. After the accident, in summer 2021 he tried walking over a few weeks but it didn't last long. Now he can't walk.
- [19] The worker testified that in terms of exercise, the longer he walks the more it hurts; the pain varies daily; and he has no treatment now. He wore a left knee brace in March 2021. The worker testified that he had an MRI and x-ray which shows nothing unusual and no smashing of bones in the left knee. He is now more of a homebody and does not go out as he used to or drive longer distances. The worker testified that in 2003 he slipped on a deck but he has had no other accidents.
- [20] The Worker's Report of Injury to the WSIB (Form 6) dated April 5, 2021 indicated the workplace injury and accident occurred on November 25, 2020. The worker reported that he slipped on a wet concrete floor and fell on his left knee straining his right thigh and hip as well. The worker reported that he first started to have problems immediately after the fall. The worker reported that he had hurt this area of his body two times before on school property. He further reported that he returned to his regular job and did not lose any time until February 2021. He also reported the following:

I mentioned my injury to my doctor shortly after. I did not take forms to him and discuss my injury until after Christmas, as I assumed it would improve but by January the pain was increasing so I made an appointment, saw him, filled the

forms in, and sent them to HR. I was told at that point, early February, to leave work until the case was sorted.

I was also taking a prescription my GP gave me for depression & memory loss.

- [21] The Employer's Report of Injury to the WSIB (Form 7) dated April 14, 2021 indicated the following:

EE completed incident report form on 11/25/2020 and confirmed he did not lose time or seek medical attention. EE went off of work 02/22/2021 and the medical which was submitted came in for an absence which did not report a WSIB claim. EE informed to the employer on 04/13/21 that they are reporting for a WSIB incident based on the medical which was sought. ER has concerns with the delay in the medical attention sought and the date of the injury as well as medical information was supporting alternative general nature of illness for the absence.

- [22] Based on worker's testimony, the Form 6 and Form 7, we accept the worker had a fall at work on November 25, 2020. Thus, we find there is proof of accident.
- [23] The worker's representative submitted that the worker had a reasonable explanation for the worker delaying in seeking medical attention after the fall at work in November 2020.
- [24] The worker testified that he lost his wife due to cancer in August 2019, which was stressful for him. One additional stressor for him in this situation was that financially, he lost his second income. The worker testified that after he reported his knee symptoms his doctor recommended that he not work anymore but it was impossible for him because financially he didn't have his wife's second income. The worker testified that his doctor supported him and they sent in the papers. His doctor was sympathetic because he knew he had fallen before. The worker testified about his depression diagnosis and related symptoms, including brain fog and lack of concentration, that was brought on by his wife's health condition and their financial difficulties.
- [25] We find that there was a delay in the worker seeking medical attention and making a claim to the WSIB for a work injury arising from the November 25, 2020 slip and fall. The Form 6 was not filed until April 2021, more than four months after the November 2020 slip and fall. We noted the worker's testimony that he called Dr. Hall's office but that the doctor was on vacation. We also noted that after the accident in November 2020, the worker did not have physiotherapy or chiropractic treatment.

[26] We noted that Dr. Ron Hall completed a form for the employer's disability benefits insurer for "Other Conditions" dated April 11, 2021. Dr. Hall diagnosed the worker with "OA [left] knee" and under "Secondary and/or Complications" he indicted "fall onto [left] knee". Dr. Hall indicated that the first visit to him pertaining to this condition was on December 9, 2020 and the worker had his first absence on February 19, 2021. Dr. Hall noted that the worker had "Pain [left knee 8/10, 10 Max at times, constant with flares, difficulty walking, sitting". We note the worker went back to his regular duties at work. The worker testified that after the accident his left knee did not hurt until after a few days or a week, he did not seek medical attention until March 2, 2021, and he did not go to a walk in clinic because he had already fallen before and thought he could cope with it and he already had pain pills. We note that Dr. Hall indicated that the first visit to him pertaining to this condition was on December 9, 2020. There is an inconsistency between Dr. Hall's reporting and the worker's testimony.

[27] We note that in the CSR Eligibility Template the Eligibility Adjudicator recorded information from a call with the worker and noted the following was reported by the worker:

- "I spoke to my doctor [Dr. Ron Hall] shortly after the injury, but I didn't think it was going to be that serious."
- "I have fallen before at work and nothing really came of it. I figured it would get better and the pain would go away on its own."
- "I didn't seek proper medical attention. I had fallen previously and nothing really happened. I have since discussed the injury with my doctor in detail and he will be completing reports."

[28] Thus, we find based on Dr. Hall's reporting and the worker's reporting in the Template that the worker engaged with Dr. Hall on December 9, 2020 and physiotherapy was recommended but the worker delayed in filing an accident claim with the WSIB until April 2021.

[29] The worker's representative submitted that the hospital records substantiate the worker's testimony that he had pre-existing knee conditions but that his fall at work when he slipped on water, aggravated and exacerbated them. It was further submitted

that the hospital records show that the worker was not in therapy for several months prior to his accident. The worker's last month of treatment was May 2020, approximately six months prior to his fall, which is consistent with his testimony that he was virtually symptom free immediately prior to the fall.

- [30] The employer's representative submitted that the worker reported the fall the same day but not the injury. The worker had two WSIB claims in 2016 and 2018 respectively for bilateral knees which were fall related. After the November 2020 accident, it was submitted that the worker continued performing his regular duties and there was a three month delay in the worker claiming a work injury. The worker had a long history of depression. The employer submitted that the thin skull does not apply to the worker but rather the crumbling skull applies.
- [31] We note the thin skull doctrine states that you must take the victim of an accident as you find them, and a pre-existing condition is not a bar to entitlement, and as long as the accident made a significant contribution to the disability, the disability results from the accident.
- [32] We have considered *Decision No. 2077/19*, 2020 ONWSIAT 33 which discusses the crumbling skull doctrine:

I have considered as well whether the minor bump on the elbow on March 12, 2015 made a significant contribution to the development of the worker's septic olecranon bursitis becoming symptomatic. It is now common place in Tribunal decisions in cases where the work activity exacerbates an underlying condition that for entitlement to be granted it must be shown the work activity made a material contribution and changed the natural course of the underlying condition. *Decision No. 2642/16* stated it as follows, which also quotes from *Decision No. 2341/08* the leading case on the subject:

The question of entitlement on an aggravation basis, in cases where there is evidence of a pre-existing or underlying, degenerative condition, such as osteoarthritis, is determined based on whether or not the work-related accident or activity has accelerated or advanced the progress of the underlying condition in a material way. The crux of the issue in this appeal is whether the workplace accident of October 8, 2010, made a significant contribution to the progression of the degenerative changes in the worker's right knee and resulted in a permanent aggravation of the condition in his right knee.

In *Decision No. 2341/08*, the Vice-Chair reviewed the Tribunal's jurisprudence on the issue, and provided the following commentary on the case law:

As I interpret the foregoing discussion, the essential requirement for entitlement for an aggravation injury is that there be evidence that a workplace injuring process has, in a material way, advanced the pathology of the pre-existing condition. If the underlying pathology is materially advanced, this constitutes injury. However, merely making an underlying condition painful or noticeable does not constitute injury. It is the change in the underlying pathology that constitutes an injury by aggravation. The presence of new symptoms may suggest advancement of the underlying pathology but may also suggest that the underlying condition has become more noticeable because of certain activities. In the end, the medical evidence must determine the outcome.

In my view the above analysis can be applied in this case. I have already found that the worker's septic olecranon bursitis was underlying and not caused by the work accident. The question then is whether the work accident caused the septic olecranon bursitis to become symptomatic. After the March 12, 2015 bump to the elbow the worker sought medical treatment and was off work. In the temporal sense then the bump certainly was close in time to the medical treatment and the need for lost time from work is also closely associated in time with the bump to the elbow. However, I find the work-related bump to the elbow served to unmask the underlying septic olecranon bursitis but did not change or advance the underlying condition of the septic olecranon bursitis. The condition was present despite the absence of medical treatment as evidenced by the worker's own description of his pre-existing symptoms, an elbow lesion that would scab but not heal and discharge fluid. The natural course of an infection is to worsen unless treated. I find in this case it is more likely than not, on the balance of probabilities, that the worker's underlying elbow septic olecranon bursitis would have become symptomatic regardless of the minor bump to the elbow. I find that this is a case more akin to that of a crumbling skull and that the thin skull theory is not applicable. The thin skull versus crumbling skull concepts are best articulated in the Supreme Court of Canada decision *Athey v. Leonati* [1996] 3 S.C.R. 458, as quoted in Tribunal *Decision No. 268/18*. For ease of reading I have substituted the word "tortfeasor" with the word insurer and the word "defendant" with insurer/employer and the word "plaintiff" with the word worker. It states:

(5) The Thin Skull and "Crumbling Skull" Doctrines

The respondents argued that the worker was predisposed to disc herniation and that this is therefore a case where the "crumbling skull" rule applies. The "crumbling skull" doctrine is an awkward label for a fairly simple idea. It is named after the well-known "thin skull" rule, which makes the insurer liable for the worker's injuries even if the injuries are unexpectedly severe owing to a pre-existing condition. The insurer must take his or her victim as the insurer finds the victim, and is therefore liable even though the worker's losses are more dramatic than they would be for the average person.

The so-called "crumbling skull" rule simply recognizes that the preexisting condition was inherent in the worker's "original position." The insurer/employer need not put the worker in a position better than his or

her original position. The insurer/employer is liable for the injuries caused, even if they are extreme, but need not compensate the worker for any debilitating effects of the pre-existing condition which the worker would have experienced anyway. The insurer is liable for the additional damage but not the pre-existing damage: ...

Likewise, if there is a measurable risk that the pre-existing condition would have detrimentally affected the worker in the future, regardless of the insurers/employer's negligence, then this can be taken into account in reducing the overall award: ... This is consistent with the general rule that the worker must be returned to the position he would have been in, with all of its attendant risks and shortcomings, and not a better position.

- [33] We find that the worker's osteoarthritis in his left knee is a pre-existing condition because the hospital records were showing that the worker was already being treated for his knees and hips (the last GLAD exercise session was in March 2020) prior to the November 2020 accident. We find the worker was not symptom free in the year prior to the accident. Based on his testimony, the worker's pain symptoms regarding his left knee varied. He also had an accommodation at work (regarding climbing stairs) prior to the November 2020 accident. While the worker's representative submitted that the worker stopped treatment in May 2020 we note that the worker testified that he missed his last session of a 10 week exercise program in March 2020 because of the pandemic and that the pandemic also contributed to him not being able to swim. The worker had also used a cane and a walker prior to the November 2020 accident. However, we find the worker did not seek further accommodation after the date of the accident in November 2020. We find the worker did not lose time from work after the November 2020 accident until he stopped working in February 2021. The worker testified that following the accident he stopped working as both his knees hurt and he indicated that after the November 2020 fall his pain varied from 4-5 or 10/10. The worker testified that his doctor told him he had arthritis and it became serious after the second fall; he has pain in his hands and he can't write or play guitar. The worker confirmed that he has had diabetes for over 20 years but it was under control.
- [34] The employer submitted that the worker accessed therapy and services prior to his workplace injury of November 2020 and that it is more likely than not that the worker's ongoing symptoms are attributed to his pre-existing osteoarthritis.

- [35] We agree because Dr. R. Hall completed forms for the employer's disability benefits insurer, "Other Conditions" dated April 11, 2021. Dr. Hall diagnosed the worker with "OA [left] knee" and under "Secondary and/or Complications" he indicated "fall onto [left] knee". Dr. Hall was specifically asked "Is this condition due to: Occupational illness/injury" and Dr. Hall checked "No". Dr. Hall indicated that the first visit to him pertaining to this condition was on December 9, 2020 and that the first date of work absence due to the condition was on February 19, 2021. Dr. Hall noted monthly physiotherapy for treatment. Dr. Hall noted that the worker also had depression secondary to grieving over his wife's recent death. In terms of prognosis, it was noted "Poor prognosis, obese, OA is expected to worsen with time". Dr. Hall noted that he recommended that the worker retire.
- [36] On the same day, Dr. Hall also completed another set of forms for the employer's disability benefits insurer, "Mental Health Conditions". Dr. Hall diagnosed the worker with "Depression" and under "Secondary" he indicated "Grieving". Under "Complicating Factors" Dr. Hall indicated "Pain Lt Knee [secondary] to osteoarthritis".
- [37] We note that in both set of forms, Dr. Hall was asked "Have you completed any other disability claim forms recently for this patient?" and he indicated "No". However, both forms go on to ask "If yes, please indicate requester: (other insurance company, CPP, QPP, Worker's Compensation Board, etc.)". Thus, we find that Dr. Hall turned his mind to the worker's left knee condition and attributed it to his pre-existing condition of osteoarthritis. Dr. Hall was explicitly asked questions on the work-relatedness of the worker's left knee condition and he indicated it was not work-related.
- [38] We give significant weight to Dr. Hall's reporting as the worker testified that Dr. Hall is his personal physician and as such, we find he has relevant knowledge about the worker's specific health condition. We note that there is no Health Professional's Report of Injury to the WSIB (Form 8) on file. The worker testified that his long term disability applications were not approved. We find that Dr. Hall's reporting suggests that the workplace accident did not materially advance the pathology of the worker's underlying osteoarthritis.

- [39] We note that the record provides information about the other WSIB claims that the worker made prior to the one under appeal in this case. A WSIB letter by an Eligibility Adjudicator dated July 31, 2017 in regards to bilateral knee injuries from a December 19, 2017 accident involving a slip and fall, which stated that entitlement for a workplace injury was denied, stated the following:

As per Policy 11-01-01, I am unable to establish proof that your current symptoms are the result of the December 19, 2017 workplace incident. You report that onset of symptoms was approximately 6-months post-incident, and initial health care was an additional month after onset. Noting the delays in onset of pain and health care, I am unable to establish proof that your current symptoms are the result of the December 19, 2017 incident. I am therefore unable to allow your claim for benefits.

- [40] We find based on the worker's testimony and the above denial of one claim, the worker has made prior WSIB claims concerning his left knee. In another knee claim, a WSIB memo dated February 2, 2004 noted the worker's left knee injury on March 14, 2003 after repetitive bending and in determining SIEF cost relief, the following was noted:

BACKGROUND: THIS WORKER SUSTAINED AN INJURY TO HIS LEFT KNEE ON 14MAR2003 AFTER REPETITIVE BENDING. I HAVE DETERMINED THIS INCIDENT TO BE OF MINOR SEVERITY.

MEDICAL OPINION: THE MC HAS REVIEWED THE PRE-EXISTING CONDITION OF OBESITY IN M#21 AND HAS DETERMINED IT TO BE OF MINOR SIGNIFICANCE THAT HAS PROLONGED RECOVERY.

SIEF APPLICATION: A MINOR ACCIDENT AND A MINOR PRE-EXISTING WARRANTS 50% COST RELIEF UNDER SIEF. THEREFORE, PLEASE APPLY 50% SIEF TO ALL CLAIMS COSTS.

- [41] Thus, we find the worker had a pre-existing condition of obesity that was medically significant in relation to the worker's left knee prior to the November 2020 accident. Dr. Hall's April 11, 2021 reporting noted that the worker's prognosis was noted to be poor due to obesity and that the osteoarthritis was expected to worsen with time.
- [42] Moreover, we also considered the Medical Discussion Paper in the file on "Osteoarthritis", November 2008, by Dr. Marvin Tile, orthopaedic surgeon, which states:

Osteoarthritis is the most common form of arthritis and indicates damage to the articular surface of a joint. It is not a single disease; rather it is the end result of a variety of joint disorders. Although inflammation is observed within the joint in the chronic course of osteoarthritis, it is not the prime cause of osteoarthritis. The distinction from true inflammatory arthritis is noted above. Although common usage

has retained the name osteoarthritis (OA); a more proper name would be *osteoarthrosis* to denote that the primary cause is not inflammatory, but due to other causes. Other names include *Wear and Tear Arthritis* or *Degenerative arthritis*.

There are many causes of OA; the two common types include:

1. Primary Idiopathic Generalized OA is of unknown etiology, but is associated with genetic or familial factors, occurring mostly in the older age group, usually more common in women and involving the small joints of the hands and feet, as well as the large joints.
2. Secondary OA has a known etiology, the degenerative joint disease occurs in response to a recognizable local or systemic factor. For our purpose, those caused by mechanical factors, such as deformity or malalignment in the limb, or injury to a joint are most relevant.

...

3. Does the presence of osteoarthritis in the x-ray of an asymptomatic patient suggest that he or she will likely become symptomatic?

This disease is often, but not always, slowly progressive on x-ray; and in those cases, the patient may become aware of pain and stiffness in the involved joint. The individual clinical response to the presence of OA on x-ray and the degree of disability varies markedly.

4. What factors might precipitate the onset of symptoms of in osteoarthritis?

Episodes of inflammation in the joint may precipitate the onset of symptoms. Trauma to the joint may also initiate symptoms

[43] This Medical Discussion Paper also notes that “Obesity is a factor in the progression of symptoms. Recent studies have shown that increased body mass has a greater effect on the knee joint, and little effect on the hip.”

[44] Furthermore, the worker was 68 years old at the time of the accident. As noted above the worker’s obesity was considered a pre-existing condition in relation to his left knee back in 2003 and it was considered relevant by Dr. Hall in April 2021. The worker had prior left knee WSIB claims. We find based on his testimony that the worker was in physiotherapy for his left knee (the GLAD exercise program in March 2020), and he was using anti-inflammatories and pain killers for his left knee prior to the November 2020 accident. The Regional Hip and Knee Replacement Program note with a fax date of May 31, 2019 noted the referring doctor as Dr. Ron Hall and the diagnosis was left knee osteoarthritis. Thus, we find the worker’s left knee osteoarthritis is a pre-existing condition and it was not caused by the November 2020 accident.

- [45] The presence of a pre-existing condition makes relevant provisions found in Operational Policy Manual Document No.11-01-15. That document, entitled “Aggravation Basis”, stipulates that entitlement for an aggravation injury is generally limited to the period during which the worker experiences acute injury with benefits continuing “until the worker returns to the pre-accident state”. The policy provides guidelines for determining the extent to which an aggravation injury contributes to subsequent disability. The policy also recognizes that an aggravation injury can result in permanent impairment. The policy states on this point:

In some cases, workers never return to the pre-accident state. If there is a permanent worsening of the pre-accident impairment, the decision-maker may determine that the work-related injury/illness has permanently aggravated the pre-accident impairment. If medical evidence confirms that the work-related injury/illness permanently increased the worker’s pre-accident impairment, the worker may be entitled to a non-economic loss benefit.

- [46] Finally, we note a summary of the Tribunal case law in this area, from *Decision No. 1592/01*, 2001 ONWSIAT 2556, at paragraph 21:

It is now commonplace in Tribunal case law that for entitlement to succeed on an aggravation basis, one must be satisfied that the work duties or a work incident changed the natural course of the underlying condition.

- [47] We interpret the above as indicating that medical evidence must establish on a balance of probabilities that the workplace injuring process has, in a material way, advanced the pathology of the pre-existing condition. If the underlying pathology is materially advanced, this constitutes injury. However, merely making an underlying condition painful or noticeable does not constitute injury. It is the change in the underlying pathology that constitutes an injury by aggravation. The presence of new symptoms may suggest advancement of the underlying pathology but may also suggest that the underlying condition has become more noticeable because of certain activities. In the end, the medical evidence must determine the outcome.
- [48] We considered whether the November 2020 slip and fall aggravated the worker’s osteoarthritis. However, we note that the worker testified that after his first fall he went to a clinic and had some relief with some exercises. We note that after the November 2020 fall, the worker did not engage with his doctor until December 9, 2020.

Based on the other WSIB claims reported in the file, we find the worker has sought medical attention at a hospital or a clinic in the past when he claimed a workplace injury. Based on his experience, we find the worker knew the importance of seeking medical attention if he believed he suffered a workplace injury.

- [49] We find the worker was symptomatic in his left knee in the year prior to the accident. The worker's representative submitted that the worker was last symptomatic in May 2020. As per the worker's testimony, his knee symptoms varied prior to the accident. According to the Medical Discussion Paper, "Osteoarthritis", "In the large joints, especially in the lower extremity (hip, knee), the symptoms may become progressive and disabling." It further states:

Pathologic features

No matter what the cause, be it primary osteoarthritis or secondary, once a defect occurs in the articular surface of the joint, the degenerative process may ensue. Cracks may occur around the defect, the defect may deepen and the articular cartilage lost. The underlying bone then becomes the joint surface, the surface becomes polished or eburnated³. This surface is less suitable than articular cartilage for joint movement. The joint and the bone beneath it respond by increasing density in the subchondral bone under the defect and osteophytes or spurs occur along the joint margin. At a later stage, the resulting osteoarthritic joint has large defects in the articular cartilage, thicker bone in the subchondral area, osteophytes (spurs) on the joint margin and in most cases, the joint becomes stiffer. At times during the progressive degenerative process inflammation may ensue, causing acute flare-ups of symptoms.

- [50] We find that the worker's symptoms after the November 2020 fall were ongoing symptoms related to his pre-existing degenerative osteoarthritis because the hospital records in the file establish that the worker was in the GLAD exercise program in the year prior to the November 2020 accident and it is consistent with the Medical Discussion Paper which indicates that with osteoarthritis there is a "progressive" degenerative process inflammation that occurs, "causing acute flare-ups of symptoms". The Total Joint Assessment Clinic Referring Physician Letter dated May 22, 2019 noted that the worker was seen at the clinic for his left knee and a surgical consult was not recommended yet because additional conservative measure "may be beneficial" including injections, medication, weight loss, bracing and the GLAD program. The Regional Hip and Knee Replacement Program note with a fax date of May 31, 2019 noted the referring doctor as Dr. Ron Hall and the diagnosis was left knee osteoarthritis.

According to hospital records, the worker was accessing treatment (GLAD exercise program) at the hospital with the initial GLAD assessment on January 24, 2020 for pain relating to his knees and hips. The hospital records show that his last GLAD exercise class was in March 10, 2020. The March 10, 2020 record noted “variable pain [and] symptoms – hips often bothering [the worker]” and it was queried “? [greater] than knee now”. Dr. Hall indicated that the worker’s first absence due to his condition was February 19, 2021. After the accident, the worker returned to regular duties, which included an accommodation that the employer had provided prior to the November 2020 accident. Even if the worker began experiencing increased pain after the November 2020 accident, we find it is not work-related based on Dr. Hall’s reporting.

- [51] Based on Dr. Hall’s reporting, we find that the worker’s left knee condition is not work-related and is related to pre-existing osteoarthritis. We find that the medical evidence does not establish on a balance of probabilities that the workplace injuring process has, in a material way, advanced the pathology of the pre-existing condition. Furthermore, the worker’s symptoms related to his left knee after the accident are more than likely due to his pre-existing osteoarthritis.
- [52] We note that Dr. Hall did not mention the worker’s hip and thigh. Thus, we find the worker does not have a personal work related injury to his right hip or right thigh because there is apparently no diagnosed impairment.
- [53] We find, on a balance of probabilities, that the November 2020 accident did not make a significant contribution to the worker’s left knee, right hip or right thigh condition.
- [54] In conclusion, on a balance of probabilities, we find that the medical evidence establishes that the worker’s osteoarthritis in his left knee is not compatible with the mechanism of injury. Based on the medical evidence, we find the worker’s left knee osteoarthritis is a pre-existing degenerative condition. We also find that the workplace injury did not in a material way, advance the pathology of the pre-existing condition. It is more likely than not that, given the worker’s age (68 years old at the time of the accident) and his obesity, the worker’s osteoarthritis would have resulted in symptoms of pain regardless of the accident.

[55] There is no persuasive evidence that the November 2020 work accident caused the worker's osteoarthritis. At the time of the accident the worker was 68 years old, the worker has a history of left knee problems, he was diagnosed with osteoarthritis in 2019, and his left knee symptoms were ongoing in March 2020. We also considered whether the mechanism of injury aggravated the worker's pre-existing osteoarthritis and we find that the preponderance of the medical evidence does not indicate that the mechanism of injury caused an aggravation or exacerbation of the worker's pre-existing left knee osteoarthritis. There is no diagnosis for the worker's right hip and right thigh after the November 2020 accident. Consequently, the five points required under Operational Policy Manual Document No. 11-01-01 to allow a claim for left knee osteoarthritis have not been established.

DISPOSITION

[56] The appeal is denied.

DATED: September 18, 2025

SIGNED: V. Patel, M.P. Moreau, J. Mandoko